co-op.care Execution Package

Friday, February 27, 2026

🔴 IMMEDIATE: Before Your 2 PM Call

Status Update

- **Pavel CAN'T make the 2 PM call** — has a conflict. Shared his booking calendar. Available after 2 PM PT most days.
- **Jacob replied this morning (10:36 AM)** — infrastructure makes sense, can start Monday/Tuesday, needs doc access, has 3 open questions, SOW coming
- **Today's 2 PM call is now Blaine + Jacob only**

Action Items (your 5-minute tasks):

1. ☐ **Grant Jacob access to the 25-page architecture doc** (he requested access, hasn't been able to read it)
2. ☐ **Send the reply to Jacob** (drafted below — answers his 3 questions)
3. ☐ **Send the reply to Pavel** (drafted below — schedules separate call)
4. ☐ **Show up at 2 PM** with the meeting prep below

EMAIL 1: Reply to Pavel (Schedule Separate Call)

To: pavel@health-samurai.io **CC:** jacob@cohesionxlabs.com **Subject:** Re: Invitation: co-op.care Phase 1 Kickoff

Pavel,

No problem at all. Jacob and I will align on Phase 1 scope today, then I want to get you on a call early next week to lock Aidbox integration specifics.

Three things I'd like to cover with you:

1. **Aidbox pricing for our use case** — we're starting with a single-tenant FHIR R4 instance for the Boulder cooperative, scaling to Multibox multi-tenant as we federate. What does the licensing look like at each stage?
2. **BAA execution** — we need a signed BAA between co-op.care Technologies LLC and Health Samurai before we handle any PHI. Can you send your standard BAA template?

3. **Aidbox + Cohesion integration plan** — Jacob (CC'd) is our engineering partner building the application layer on top of Aidbox. I want to make sure the three of us agree on the integration architecture before Jacob's team starts Monday.

I'll book a time on your calendar for Monday or Tuesday. Does 2:30 PM PT work as a backup if your booking calendar doesn't have slots?

Blaine
484-684-5287

EMAIL 2: Reply to Jacob (Answer His 3 Questions)

To: jacob@cohesionxlabs.com **Subject:** Re: Co-op Care - Next Steps

Jacob,

Good. Let's lock this down today. Answering your three questions:

1. CMS ACCESS requirements for the architecture doc:

The ACCESS application requires a "Technology and Data Infrastructure" narrative section. Specifically, CMS wants to see:

- FHIR R4 API capability for interoperable data exchange with CMS and participating providers
- Electronic Visit Verification (EVV) compliance methodology
- Clinical outcome measurement system (how you track and report Outcome Attainment Threshold — 50% of aligned beneficiaries must show measurable improvement)
- Data security and HIPAA compliance posture
- Patient attribution and panel management approach
- Quality reporting infrastructure (CMS quality measures + ACCESS-specific outcome measures)

I'll send you a clean checklist of these requirements before our call today so you can validate against your quote. The architecture doc needs to tell CMS: "this is a technology-enabled care management platform with the infrastructure to receive, process, and report clinical data at the standard CMS requires."

2. BAA:

I'll execute the BAA. co-op.care Technologies LLC will be the covered entity. Cohesion signs as a business associate. I'll have the BAA template ready for your review by Monday. Standard HHS model BAA language — nothing exotic.

3. HIPAA compliance platform:

I'm evaluating Vanta vs. AccountableHQ. Vanta is the heavier lift but more credible for CMS. AccountableHQ is faster and cheaper for early stage. I'll make the call this weekend and have it set up before you start. Either way, this cost is on co-op.care, not Cohesion.

On payment terms:

50% deposit before work begins, 50% at milestone — that works. At $15K, that's $7,500 up front. I'll have the deposit wired before Monday. Please send the SOW today or tomorrow so I can sign over the weekend.

On the 25-page doc:

I'll grant you access right now. Sorry about that — should have set sharing when I sent it.

See you at 2.

Blaine

MEETING PREP: 2 PM Call with Jacob (45 min)

Call Objectives (walk in knowing exactly what you want)

1. **Confirm Monday March 3 start date ✓** (Jacob already said yes)
2. **Align on Phase 1 deliverables** — make sure Jacob's scope matches what ACCESS needs
3. **Resolve the Aidbox question** — Jacob builds ON Aidbox, not a FHIR server. Pavel call scheduled separately.
4. **Get SOW timing** — need it signed this weekend for Monday start
5. **Set the weekly cadence** — suggest Monday/Thursday check-ins for the 5-week sprint

Talking Points (in order)

Open: "Pavel couldn't make it — he and I are scheduling separately for Monday or Tuesday. Today let's lock down the Phase 1 scope between us so your team can start Monday."

Scope Confirmation:

- CII Assessment (12-slider, web UI, scoring engine, data capture → Aidbox Observation resources)
- CRI Assessment (14-factor, clinical weights, fall-risk 1.1x multiplier, → Aidbox)
- HIPAA-compliant cloud infrastructure (AWS, encrypted, audit logging, BAA)
- Production architecture document (dual purpose: engineering spec + ACCESS application narrative)
- Founding member deposit flow (Stripe integration, $100 deposits)

What Jacob does NOT build:

- FHIR server (Aidbox handles this)
- OAuth/SMART on FHIR auth (Aidbox handles this)
- HL7 v2 pipeline (Aidbox handles this — for BCH discharge feeds later)
- Terminology server (Aidbox handles this)

ACCESS requirements checklist:

- Share the checklist I've prepared (see Section 4 below)
- Jacob validates his scope covers these requirements
- Any gaps get flagged now, not in week 4

Payment:

- Confirm $15K total (engineering only)
- 50% deposit ($7,500) wired before Monday
- Variable costs (AWS ~$200/mo, Aidbox TBD, HIPAA platform ~$100-300/mo) covered by co-op.care
- SOW signed this weekend

Weekly cadence:

- Propose Monday/Thursday 30-min standups
- Week 1: Aidbox setup + infrastructure + CII prototype
- Week 2: CRI + FHIR resource models + Omaha mapping
- Week 3: NLP pipeline + billing logic + Clinical Director workflow
- Week 4: ACCESS application drafting + outcome tracking
- Week 5: Final submission + BVSD demo prep

Close: "Send me the SOW tonight. I'll sign it, wire the deposit, and your team starts Monday with clear scope, clear architecture, and clear deadlines."

SECTION 4: CMS ACCESS Model — Technical Requirements Checklist

This is what Jacob asked for. Share it with him on the call or send after.

ACCESS Application Technology Requirements

#	Requirement	What CMS Wants to See	Phase 1 Deliverable
1	FHIR R4 API	Interoperable data exchange with CMS,	Aidbox FHIR R4 instance configured with

#	Requirement	What CMS Wants to See	Phase 1 Deliverable
		hospitals, PCPs	US Core profiles
2	Clinical Assessment Tools	Standardized patient assessment and risk stratification	CII (caregiver burden) + CRI (care needs) live on web
3	Outcome Measurement	System to track Outcome Attainment Threshold (50% improvement)	CRI longitudinal tracking + Omaha KBS outcome scores
4	Electronic Visit Verification	GPS/time-stamped visit verification for home-based services	EVV module capturing check-in/out with geolocation
5	Clinical Coding System	Standardized clinical documentation	Omaha System (Problem Classification + Intervention + KBS)
6	HIPAA Compliance	Security risk assessment, encryption, access controls, BAA chain	HIPAA platform + Aidbox AccessPolicy + audit logging
7	Patient Attribution	Panel management for aligned beneficiaries	Aidbox Patient + CareTeam + EpisodeOfCare resources
8	Quality Reporting	CMS quality measures + ACCESS-specific outcomes	SQL on FHIR analytics views for measure calculation
9	Care Coordination	Communication between care team members and PCPs	FHIR CommunicationRequest + Task resources
10	Billing Infrastructure	PIN/CHI code generation for CMS claims	Claim resource generation from documented interventions

What's In Phase 1 Scope:

✅ Items 1, 2, 3, 6, 7 (core infrastructure)
✅ Architecture document covering all 10 items (narrative for CMS)

What's Phase 1+ (post-April 1):

⏳ Items 4, 5, 8, 9, 10 (production features built on the Phase 1 foundation)

Key Distinction for Jacob:

The ACCESS *application* needs to describe the architecture for all 10. The ACCESS *launch* (July 2026) needs all 10 operational. Phase 1 builds the foundation and writes the application. Phase 1+ builds the remaining production features.

SECTION 5: 33-Day Execution Tracker (Feb 27 → April 1)

WEEK 0: This Weekend (Feb 27-Mar 2)

Task	Owner	Status	Notes
Grant Jacob access to architecture doc	Blaine (2 min)	☐	He requested access, just approve it
Send reply to Jacob	Blaine (1 min)	☐	Copy email above, paste, send
Send reply to Pavel	Blaine (1 min)	☐	Copy email above, paste, send
Book Pavel call (Mon or Tue)	Blaine (2 min)	☐	Use his calendar link
2 PM call with Jacob	Blaine (45 min)	☐	Use meeting prep above
Sign Cohesion SOW	Blaine (5 min)	☐	When Jacob sends it
Wire $7,500 deposit to Cohesion	Blaine (5 min)	☐	From Elevations or personal
File LLC with Colorado SOS	Blaine (15 min)	☐	See LLC filing guide below
Choose HIPAA platform (Vanta or AccountableHQ)	Blaine (10 min)	☐	Recommendation: AccountableHQ for speed
Set up HIPAA platform account	Blaine (15 min)	☐	After choosing

WEEK 1: March 3-7 (Cohesion Sprint Begins)

Task	Owner	Status	Notes
Cohesion kickoff + Aidbox setup	Jacob/Cohesion	☐	Monday March 3
Pavel/Aidbox integration call	Blaine + Jacob + Pavel	☐	Book for Mon/Tue
Lock Aidbox pricing	Blaine + Pavel	☐	On the Pavel call
Execute BAA (Cohesion)	Blaine	☐	HHS model template
Execute BAA (Health Samurai)	Blaine + Pavel	☐	Request Pavel's template

Task	Owner	Status	Notes
CII assessment prototype on Aidbox	Jacob/Cohesion	☐	Week 1 deliverable
CareOS architecture document v1	Jacob/Cohesion	☐	First draft
Post LinkedIn article	Blaine (5 min)	☐	Cooperative economics piece (already written)
Clinical Director outreach — send 5 emails	Blaine (10 min)	☐	Drafts below

WEEK 2: March 10-14

Task	Owner	Status	Notes
CRI assessment prototype on Aidbox	Jacob/Cohesion	☐	
FHIR R4 resource models defined	Jacob/Cohesion	☐	Patient, Observation, CareTeam, EpisodeOfCare
Omaha System → FHIR crosswalk draft	Jacob/Cohesion + Claude	☐	I'll build the mapping table
ACCESS application — Section 1 draft	Claude → Blaine review	☐	Organization description
ACCESS application — Section 2 draft	Claude → Blaine review	☐	Technology infrastructure
Clinical Director — follow up on outreach	Blaine (10 min)	☐	
BCH outreach email (Grant Besser)	Blaine (5 min)	☐	Draft below

WEEK 3: March 17-21

Task	Owner	Status	Notes
CareOS NLP → Aidbox integration	Jacob/Cohesion	☐	Caregiver note → FHIR pipeline
PIN/CHI billing logic prototype	Jacob/Cohesion	☐	
Clinical Director workflow mock	Jacob/Cohesion	☐	MD/DO oversight UI

Task	Owner	Status	Notes
ACCESS application — Section 3 draft	Claude → Blaine review	☐	Clinical model
ACCESS application — Section 4 draft	Claude → Blaine review	☐	Outcome measurement
BVSD pilot proposal finalize	Claude → Blaine	☐	Already drafted

WEEK 4: March 24-28

Task	Owner	Status	Notes
ACCESS application — full draft assembly	Claude + Jacob	☐	All sections integrated
Architecture doc finalized (CMS-ready)	Jacob/Cohesion	☐	Dual-purpose document
Outcome tracking system design	Jacob/Cohesion	☐	Omaha KBS longitudinal
BCH discharge workflow mockup	Jacob/Cohesion	☐	HL7 v2 → FHIR flow
ACCESS application internal review	Blaine + Jacob	☐	Red team the application

WEEK 5: March 31 - April 1

Task	Owner	Status	Notes
ACCESS application final edits	Claude + Blaine	☐	Polish pass
ACCESS Model application submitted	**Blaine**	☐	**DEADLINE: April 1**
Production environment verified	Jacob/Cohesion	☐	CII + CRI live
BVSD demo prep	Jacob/Cohesion	☐	For employer pilot pitch

SECTION 6: LLC Filing Guide (15 minutes)

Colorado Secretary of State — Online Filing

URL: https://www.sos.state.co.us/pubs/business/businessHome.html

Entity: co-op.care Technologies LLC **Type:** Domestic Limited Liability Company **Registered Agent:** Blaine Warkentine (you can be your own initially) **Registered Agent Address:** [Your Boulder address] **Principal**

Office: [Same] **Management:** Member-managed (single member initially)

Filing Fee: $50 (online) **Processing Time:** 1-2 business days online

Steps:

1. Go to Colorado SOS business filing portal
2. Select "File a new business entity"
3. Choose "Domestic Limited Liability Company"
4. Entity name: "co-op.care Technologies LLC"
5. If name is taken, try "Co-op Care Technologies LLC" or "Coop Care Technologies LLC"
6. Registered agent: yourself
7. Principal office: your Boulder address
8. Management structure: member-managed
9. Pay $50
10. Save the confirmation number and filing receipt

After filing:

- Apply for EIN at irs.gov (free, instant online)
- Open business bank account at Elevations Credit Union
- This LLC is the entity that signs the Cohesion SOW, executes BAAs, and applies for ACCESS

Why this weekend: The LLC must exist before you can execute the BAA with Cohesion, sign the SOW as a legal entity, or begin Medicare Part B enrollment. Every day of delay cascades.

SECTION 7: Clinical Director Recruitment (5 Emails Ready to Send)

Target Profile:

- MD or DO with active Colorado license
- Interested in value-based care, cooperative economics, or aging-in-place
- Willing to serve as Clinical Director (part-time, oversight role)
- Hours: ~5-10 hrs/week initially
- Compensation: equity + stipend (negotiable), full ownership path

Email Template (Personalize the [brackets]):

Subject: Clinical Director opportunity — worker-owned home care cooperative (Boulder)

Hi Dr. [Name],

I'm building a worker-owned home care cooperative in Boulder that's applying to the CMS ACCESS Model — a 10-year federal program that pays for technology-enabled chronic care management.

ACCESS requires a physician Clinical Director (MD/DO) to oversee clinical safety and care protocols. This is a part-time oversight role (~5-10 hours/week) with equity ownership in the technology entity and a path to meaningful participation in a model that's designed to keep people at home instead of in facilities.

What makes this different from typical medical director gigs:

- The caregivers own the cooperative (W-2, $25-30/hr, equity)
- Families pay $550-3,500/month instead of $9,900/month for facility care
- The technology translates caregiver observations into clinical language (Omaha System, FHIR R4) so you see structured data, not chaos
- CMS pays $180-420/beneficiary/year for achieving health outcomes — this is a long-term federal revenue stream, not a grant

We're launching in Boulder in March with a Cohesion Health engineering team and Aidbox (Health Samurai) as our FHIR backend. The ACCESS application deadline is April 1.

Would you have 20 minutes next week for a conversation? I can share the full architecture and show you exactly what the Clinical Director role looks like.

Blaine Warkentine Founder, co-op.care Technologies LLC blaine@co-op.care 484-684-5287

Where to Send (5 targets):

1. **Boulder Community Health hospitalists** — they see the discharge problem daily. Ask Grant Besser for a warm intro, or search BCH physician directory.
2. **DPC (Direct Primary Care) physicians in Boulder** — they already reject the traditional model. Search "direct primary care Boulder Colorado" — these doctors are philosophically aligned.
3. **Colorado Academy of Family Physicians** — post on their job board or email their member services.
4. **University of Colorado School of Medicine** — faculty interested in social determinants, community health, or geriatrics. Check the Division of General Internal Medicine and the Center on Aging.
5. **Retired physicians with active Colorado licenses** — Colorado Medical Board license lookup at https://dpo.colorado.gov/MedicalBoard. Filter for active licenses in Boulder County. Retired docs who maintain their license are perfect — they want meaningful part-time work.

SECTION 8: BCH Outreach Email (Grant Besser)

Subject: co-op.care — cooperative home care for Boulder discharge coordination

Grant,

I'm launching a worker-owned home care cooperative in Boulder that solves a specific problem your discharge planners deal with every day: patients who are medically stable but have nowhere safe to go home to because family caregivers are overwhelmed and there aren't enough home care workers.

The model:

- Caregivers are W-2 employees who own the cooperative ($25-30/hr)
- Families pay $550-3,500/month instead of $9,900 for facility care
- Clinical documentation uses Omaha System coding mapped to FHIR R4 (we're building on the Aidbox platform from Health Samurai)
- We're applying to the CMS ACCESS Model (10-year federal payment program for chronic care management)

What I'd like to explore with BCH:

- Discharge coordination partnership — when a patient is ready for home but needs a care plan, we provide the caregiver, the technology, and the family support
- This reduces blocked bed days (each day costs BCH ~$2,500 in unreimbursed care)
- We're not competing with BCH Home Health — we're filling the gap between hospital discharge and independent living

We're in active development with Cohesion Health (engineering) and Health Samurai (FHIR infrastructure), launching in Boulder in March. Would you have 30 minutes in the next two weeks?

Blaine Warkentine Founder, co-op.care Technologies LLC blaine@co-op.care 484-684-5287

SECTION 9: What Claude Handles Next

Once you confirm "go" on the items above, here's what I'll build for you immediately:

1. **ACCESS application full draft** — all narrative sections, ready for your review
2. **BAA template** — HHS model language customized for co-op.care Technologies LLC
3. **Omaha System → FHIR R4 crosswalk** — the clinical mapping table Jacob needs for development
4. **BVSD pilot proposal** — final version with updated Cohesion/Aidbox architecture
5. **Convertible promissory note** — for the $7K bridge capital if needed

6. **Website update** — integrate the latest positioning, Cohesion partnership, ACCESS timeline
7. **Social media content** — next 4 weeks of LinkedIn posts aligned to the launch timeline

You focus on: grant doc access, send emails, wire money, show up to calls, file the LLC. I handle everything else.